EHC 26 9–12 December 2026 Berlin (DE) and online
20th European Headache Congress
P548

Scan me, for digital PDF version:

References

COMPARISON OF THE EFFICACY OF NARATRIPTAN AND SUMATRIPTAN FOR ACUTE MIGRAINE TREATMENT: A COMPARATIVE STUDY

Daniel Neto[1] , Arthur Soares[1], Norma Espindola[1], Prof. Dr. Wagner Horta[1]
[1] *Universidade de Pernambuco, Faculdade de Ciências Médicas, Recife, Brazil*

Background and Aims

Migraine is a prevalente and disabling neurological disorder, currently recognized as the second Leading cause global disability. Triptans such as sumatriptan (SMT) and naratriptan (NRT) are selective 5-HT1B/1D receptor agonists and constitute first-line pharmacologic agentes for abortive migraine therapy. SMT was the first triptan introduced and marked a major therapeutic milestone. However, its limitations include low oral bioavaliability, high recurrence rates, and contraindications in patients with coronary artery disease. NRT, a second-Generation triptan, has emerged as a potentially safer and more durable option.

Objective

This study aims to compare the efficacy and tolerability of oral SMT and NRT in the acute treatment of migraine.

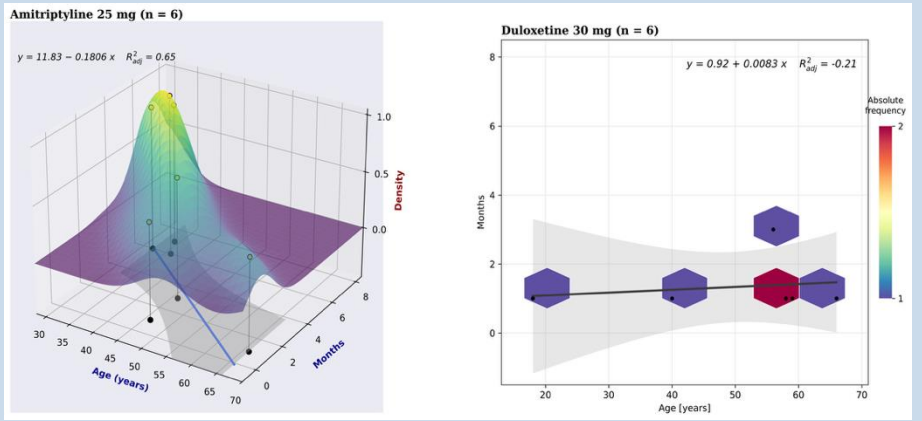
Methods

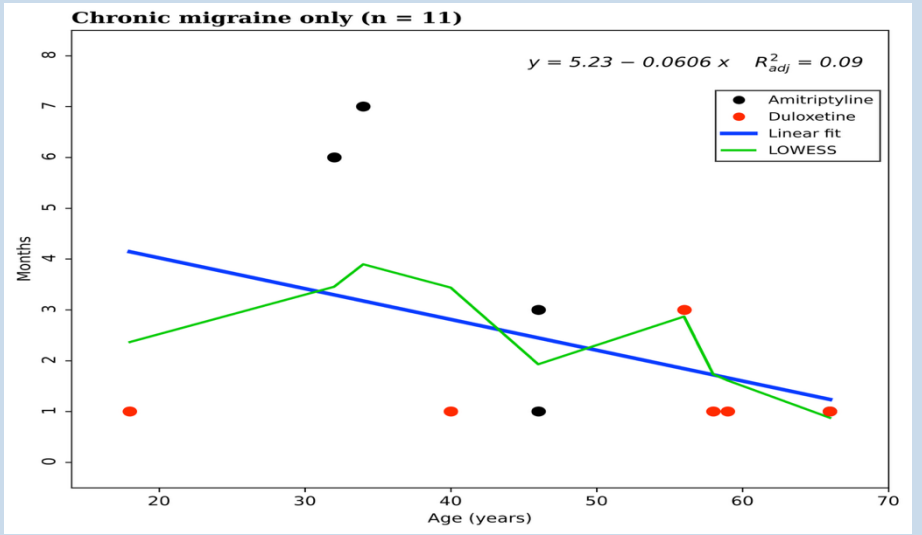
An integrative literature riview was conducted using the ScIELo, BVS and PubMed databases: Articles published in English and Portuguese over the past five years were selected, based on predefined inclusion and exclusion criteria. Keyworrds included “Migraine”, “Prophylaxis”, “Sumatriptan”, and “Naratriptan”. Studies were analazed with a focus on efficacy, onset of action, recurrence rates, and adverse events profiles of oral SMT and NRT.

Results

SM 100 mg showed faster onset, wit 45% of patients achieving pain freedom at 4 hours, versus 31% with NRT 25mg. At higher doses, NRT 10 mg demonstrated comparable efficacy (65% pain freedom at 4 hours) with longer duration of relief.

Accoding to recente clinical evidence, NRT also shows a lower recurrence rate and reduced need for rescue medication. Adverse event rates were lowest with NRT 2,5 mg, although Higher doses (7,5-10,g) increased adverse effects, they remained lower than those observed with SMT. These findings suggest a clinically meaningful Advantage for NRT in pacientes requiring sustained relief and better tolerabillty.





Conclusions

Both SMT and NRT are effective oral agentes for abortive migraine treatment, each with distinct pharmacodynamic Properties. NRT may offer superior tolerabillity and sustained efficaccy in selected patients. These insights support the need for individualized therapeutic strategies based on patient profile, clinical priorities, and cardiovascular risk.